



# For care homes

A plan for a 15–25 room care home — and what we deliberately leave out.

**A PLAN** This is a system being built, not a product you can buy. What runs today and what does not is at the bottom of page two.

## Before anything else — this is not a nurse call system

A care home needs a **proper, certified nurse call system**.

Our system is designed to sit **ALONGSIDE** one, not to replace it. It cannot be the only way of calling for help, and we would not want it to be.

Certified systems are available off the shelf. We are happy to help compare them — but we will not build one, and we will not pretend that what we build is one.

### Who it is for

The large institutional systems exist, they are good, and they are expensive. What a home of a few dozen rooms usually gets offered is something it does not need, at a price it cannot carry.

Within a few weeks, **two privately run homes** asked about this. One cares for people living with dementia; the other for older people who are cognitively well but can no longer manage on their own.

One of the homes offered to **help the development with advice and experience**.

### What we would build

| WHERE                | WHAT                                                                                       |
|----------------------|--------------------------------------------------------------------------------------------|
| Every room           | A wall console with one large help button. Optionally a wearable button too.               |
| Every room           | A medication confirmation button, and the record behind it.                                |
| At the nurse station | A large screen showing every room at once, with sound. At more than one station if needed. |
| In the building      | One small computer that holds the records. Nothing leaves the building.                    |

### The medication record

**We never display “taken”.** The system does not know that. It knows somebody pressed a button — so it says “confirmed at 08:12 by ...”, or “not confirmed”.

**Append-only.** Entries are never edited and never deleted; a correction is a new entry, and both stay visible. This is how clinical records work.

**Who and when.** Every administration records the resident, the round, the member of staff who confirmed it, and the exact time. Identification by tap fob, two seconds.

**This is for staff, not against them.** When something is questioned weeks later, a timed record is what shows the round was done.

### What this system does NOT do

- **Not a nurse call system**, and it must not be used as one.
- **Not a medical device.** It does not dispense, measure or diagnose.
- **Does not replace the medication round** or a nurse’s judgement.
- **Does not watch residents.** There are no cameras and no microphones.
- **Not a life-safety system**, and nobody should be told that it is.

## The rule we build by

A certified nurse call system watches its own wiring: cut a cable and it raises an alarm. Ours does the equivalent in software — **every console reports in continuously**, and if one goes quiet the nurse station says so plainly, in words, and out loud.

**Silence must never look like “everything is fine”.** This is the single most important rule in the whole design, and it is the reason we can offer the system at all.

Related: **colour is never the only signal**. Anything important is shown as text and shape as well — from across a corridor, and for someone who is colour-blind.

## No two homes are alike

Two homes, two almost opposite problems — on the same hardware. Here we need the homes’ expertise far more than they need ours.

### Where people live with dementia

- Someone living with dementia often **cannot call for help** — not because the button is out of reach, but because it does not occur to them that it exists. The valuable signal is often the *absence* of something.
- The screen in the room shows as little as possible: one button, no changing text.
- Fully dark at night — a screen must not light a bedroom — with the button still working.

### Where residents are cognitively well

- They want more, not less:** messages, appointments, reminders, an “I’m all right” tile.
- Privacy weighs heavier here: someone who thinks clearly has a right to know what the system records about them — and a right to say no.
- Same hardware, different setting.** Not two products, but one that the home configures for itself.

## Where we are today

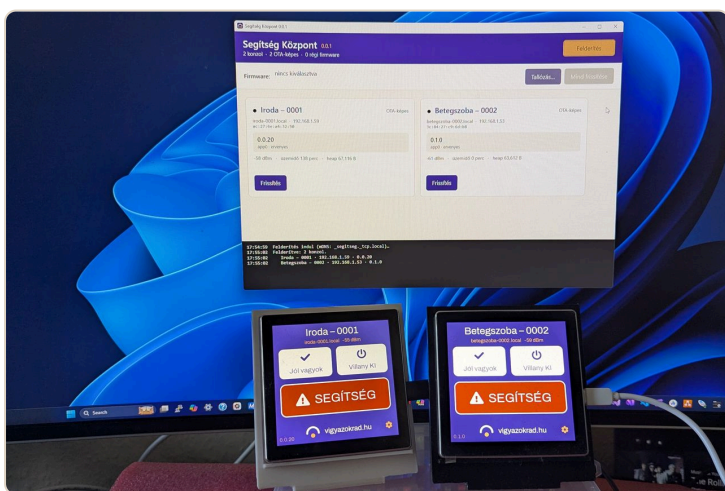
### What runs today, and is measured

- The wall console: a large HELP button, two tiles, its own network name.
- The consoles speak to each other: the call appears in the other room and the “I’m coming!” reply gets back. Measured with two real panels: **ten seconds**.
- Updates over the network, with no cable — and if one goes wrong, the device returns by itself.
- The Segítség Központ at **0.0.1**: discovery and updating.

When the first installation comes, it starts with **a single room**, not fifteen. One room, a few weeks, and then questions — before a hole is drilled anywhere else.

### What is not there yet

- The medication record and fob identification.
- The full nurse station screen and the sound.
- The small computer that holds the records.
- Any installation in a real home. **That has not happened yet.**



The nurse station program today (Segítség Központ 0.0.1) and the two consoles it can see. It finds them on the network by itself and updates them with no cable.

If you run a home, work in one, or know one from the inside as a relative: every objection is more useful now than later.

The plan in detail, in three languages: [vigyazokrad.hu/otthonoknak.html](http://vigyazokrad.hu/otthonoknak.html) · [info@vigyazokrad.hu](mailto:info@vigyazokrad.hu)

20 September 2026 · Kürtösi Péter · Vigyázok Rád